

Susan Allen — Current Site Audit

(susanallentherapy.com)

Audit conducted 2026-07-13 against the WFC 6-gate Critique Heuristics framework. For pre-kick-off reference (Tue 7/14 10:00 AM Zoom with Susan — Milestone 1 of the 8-hr Phase 1 plan).

Summary

Gate	Result	Severity
1. Element Count	⚠️ FAIL (about page)	HIGH
2. Arousal Calibration	⚠️ MIXED (H1 ALL CAPS, hierarchy chaos)	MEDIUM
3. Revisability	✅ N/A (no forms)	—
4. Dominance Preservation	✅ PASS	—
5. Implicit Aesthetic	❌ FAIL (H1 = brand, not content)	HIGH
6. Accessibility	🟡 LIKELY PASS (needs deeper WCAG pass)	LOW

Overall: 2 HIGH fails, 1 MEDIUM flag. Site is functional but does not pass the WFC audit. Phase 1 is justified.

Quick reference — what I observed

- **Platform:** Squarespace (footer credits "POWERED BY SQUARESPACE")
 - **Pages actually present:** HOME, ABOUT, FOCUS, THERAPY, CONTACT, RESOURCES (= /blog, 3 posts)
 - **Pages referenced in dossier but NOT present:** `/services` — silently redirects to home (not a 404 as the dossier said; arguably worse because users don't know they didn't get the page)
 - **ASRM/PSI credentials** are real and accurate but buried in body text on About
 - **Photography:** Single professional headshot (about page)
 - **Color palette:** Predominantly white/cream with dark text. Neutral. Not particularly warm or specialty-tuned.
 - **Typography:** Display sans-serif for H1/H2 (caps), body sans-serif. Hierarchy is inconsistent.
 - **No CTA buttons visible** — "Schedule a Consultation" is an H2, with the actual CTA as plain links ("Consultation", "contact me")
 - **No FAQ, no resources page, no booking** (consistent with SOW out-of-scope items)
-

Gate 1 — Element Count Check (9–17 per visible screen state)

Evidence: T2-004, Principle 1 (Sensory Safety)

Threshold: 9–17 elements per visible screen state. Beyond 17, cognitive load exceeds WFC's target.

Home page (counted from snapshot)

- 1 H1 ("SUSAN ALLEN LMFT | PSYCHOTHERAPY")
- 6 nav items
- 2 H3 ("Therapy for Infertility...", "Therapy for Postpartum...")
- 1 H2 ("Schedule a Consultation")
- 1 CTA link ("Consultation")
- 1 secondary link ("contact me")
- 1 H3 ("Recent Articles")
- 3 article links
- 1 footer + 1 footer link
- Total: ~17 elements — at the upper limit, BORDERLINE PASS

About page

- 1 H1, 6 nav, 1 image, 1 H3 ("My Practice")
- ~8 body paragraphs
- 4 H3 sections (Education, Affiliations, Media, Training)
- 2 outbound links (Huffington Post articles)
- Psychology Today badge
- Footer
- Total: 25+ elements — FAIL. Progressive disclosure needed.

Focus page

- 1 H1, 6 nav
- 1 image (IRH logo)
- 1 H3 + 8+ bulleted items per specialty section
- Multiple specialty sections (Infertility, Pregnancy/Postpartum, Donor Conception, Parenting, Mid-Life, Loss/Grief)
- Total: 30+ elements — FAIL. Information density far exceeds the 9–17 threshold.

Therapy / Contact / Resources

- Not counted in detail; Resources page is the blog (3 posts).
- Contact page snapshot to be added.

Resolution per heuristics.md: Reduce visible elements through progressive disclosure. Focus page is the worst offender — needs to break into multiple pages (one per specialty, or a "specialties hub" with progressive disclosure).

Gate 2 — Arousal Calibration

Evidence: T1-002 (LPP), T1-004 (PAD coordinates), Principle 4 (Emotional Calibration)

Target PAD for therapist home page: Pleasure 1.8, Arousal -0.3, Dominance 1.0 (per

[Work/WFC/studio/page-archetype-landing](#))

Findings

Check	Result	Note
Target PAD defined for archetype?	✗ NOT DOCUMENTED	No PAD mapping in the site itself
Color palette matches low-arousal target?	🟡 PARTIAL	Neutral palette; not warm; not anxiety-triggering
No surprise motion or auto-playing media?	✅ PASS	None observed
Typography does not create visual urgency?	✗ FAIL	H1 is ALL CAPS ("SUSAN ALLEN LMFT PSYCHOTHERAPY") — uppercase at this size reads as institutional/formal, not calm
Images support emotional intent?	🟡 NEUTRAL	Professional headshot, no clinical/spa imagery
Hierarchy chaos	⚠️ FLAG	About page uses H3 as top heading; H1 reserved for brand. Inconsistent.

Key issue: The ALL-CAPS H1 ("SUSAN ALLEN LMFT | PSYCHOTHERAPY") reads as **institutional, not warm**. For reproductive + perinatal mental health (where users are often anxious, overwhelmed, cognitively depleted), an institutional voice increases arousal at the worst possible moment.

Resolution: Move H1 to be the page-specific content, not the brand. Brand goes in the header/nav. Use mixed case. See WFC's [Work/WFC/studio/tokens-typography](#) for serif display recommendations.

Gate 3 — Revisability Audit

Evidence: T1-008, Principle 2 (Dominance Through Revisability)

Findings

Check	Result	Note
Every form has draft state	N/A	No forms on site (consult intake is email-only per SOW — by design)
Every CTA has outcome description	 PARTIAL	"Schedule a Consultation" describes action; "contact me" does not
Every action has undo or return path	 PASS	Standard back button / nav
No irreversible actions without confirmation	 PASS	None
Multi-step processes show progress	N/A	None

Resolution: For Phase 1, low priority. Revisability is mostly a future concern if Susan adds online intake later. Worth noting the "contact me" link text could be more descriptive ("email Susan to schedule a consultation").

Gate 4 — Dominance Preservation

Evidence: T1-003, Principle 2 (Dominance Through Revisability)

Findings

Check	Result	Note
No urgency language or timers	 PASS	None observed
No dark patterns	 PASS	None observed
Exit options equally visible	 PASS	Nav always present
User controls pace	 PASS	No auto-advance
Pricing transparent	N/A	No pricing on site (intentional for therapy intake)

Resolution: None. This is the cleanest gate on Susan's current site.

Gate 5 — Implicit Aesthetic Scan (P2/LPP proxy)

Evidence: T1-001, T1-002, Principle 3 (Implicit Aesthetic First)

Findings

Check	Result	Note
Layout stable on first render	✅ PASS	No shifts observed
Grid system provides clear structure	🟡 PARTIAL	Simple 2-column on home; single column on About
Balance and symmetry intentional	🟡 PARTIAL	Centered layout, but H1+nav eats vertical real estate
Visual hierarchy clear within 400ms	❌ FAIL	H1 is the brand, not the content — users scan past the brand looking for the actual content
Cohesion (unified system)	🟡 PARTIAL	Looks like a single Squarespace template, but customizations feel ad-hoc
Order (predictable, scannable)	🟡 PARTIAL	Nav is consistent; section ordering varies by page
Complexity (matches audience capacity)	❌ FAIL	Focus page overshoots significantly
Simplicity (unnecessary noise removed)	🟡 PARTIAL	Some decorative elements (logo bar, Psychology Today badge) compete with content

Key issue: H1 = brand, not content. This is a significant hierarchy failure. Users who land on a page and see the brand as the largest text element don't know what the page is about. The visual hierarchy should prioritize page content over site identity.

Resolution: Move brand to header (already there). Use H1 for the actual page topic. This is one of the highest-impact changes for Phase 1.

Gate 6 — Accessibility Baseline

Evidence: WFC token specs, WCAG AA standards

Findings (initial, not full audit)

Check	Result	Note
WCAG AA contrast (4.5:1)	🟡 LIKELY PASS	Dark text on light background — likely meets threshold
Focus indicators visible	? UNKNOWN	Needs keyboard nav test
All images have descriptive alt text	🟡 PARTIAL	Snapshot shows some images with alt, some without
All forms have labels	N/A	No forms

Check	Result	Note
<code>prefers-reduced-motion</code> respected	? UNKNOWN	No motion observed; Squarespace defaults generally OK
Page navigable by keyboard	? UNKNOWN	Needs test
Screen reader announces state changes	N/A	No dynamic states

Resolution: Add a full WCAG pass to Phase 1 Milestone 5 (on-page SEO + QA, ~0.75 hr). Flagged in agenda already.

Top 3 issues to call out at kick-off (in priority order)

1. The "Services" 404 isn't a 404 — it's a silent redirect to home. Users who type `/services` (or follow an old link) get dumped to home with no indication. They probably leave. → Phase 1 fix: build the Specialties page.
2. H1 is the brand on every page, not the content. Users landing on the About page see "SUSAN ALLEN LMFT | PSYCHOTHERAPY" as the largest text — they don't know they're on the About page until they scroll. → Phase 1 fix: H1 = page content, brand stays in header.
3. ALL CAPS H1 reads as institutional, not warm. Susan's specialty (reproductive + perinatal mental health) calls for a calm, nurturing aesthetic. The caps H1 fights that. → Phase 1 fix: Mixed case + WFC Calm Base palette (peachy cream background, sage green accent).

Secondary issues (not in scope for Phase 1 unless low-cost)

- About page: 25+ elements (Gate 1 fail)
- Focus page: 30+ elements per specialty area (Gate 1 fail)
- Hierarchy inconsistency: H1 brand on most pages, H1 content on Focus; H3 as top heading on About
- Credentials buried in body text: ASRM + PSI should be visually prominent (this is Susan's biggest differentiator per [leads/wfc-susan-allen/dossier > WFC-Fit-Analysis](#))
- Color palette: neutral but not warm — misses an opportunity to communicate specialty depth

What works (don't break in Phase 1)

- Squarespace platform — staying per SOW
- Clean nav structure (6 items, all functional)

- Professional headshot
 - Psychology Today verification badge — keep this
 - Clear contact path (even if just email)
 - Real credentials (ASRM, PSI, Donor Conceived Community) — surface them more in Phase 1
 - Tone of About/Focus copy — clinical but warm in voice. Don't replace; restructure.
-

Screenshots

All captures in `./screenshots/current/`:

- `01-home-desktop.png`
- `02-about-desktop.png`
- `03-focus-desktop.png`
- `04-therapy-desktop.png`
- `05-contact-desktop.png`
- `06-resources-desktop.png` (the blog at /resources)
- `07-services-404.png` (shows the silent redirect to home)

Audit outputs feeding into kick-off conversation

- This audit fills Milestone 1 (Audit) in the [leads/wfc-susan-allen/kick-off-agenda-2026-07-14](#) agenda
- Pair with [leads/wfc-susan-allen/audit/template-shortlist](#) to walk Susan through what a Squarespace template refresh could look like
- The top 3 issues above are concrete, evidence-grounded starting points for the conversation